BASELINE → TREATMENT → FOLLOW-UP

My treatment, tests and checks

Routine blood safety checks are different from tests ordered for a symptom and from scans used to assess the cancer. Your team supplies exact dates and explains results.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ tests	Before starting: full blood count, kidney and liver tests, and dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) assessment before first fluorouracil unless already tested. electrocardiogram (ECG) is needed for compromised heart function; hearing assessment and measured kidney clearance are arranged when indicated. Discuss hearing, tinnitus, nerve symptoms, swallowing, weight, medicines and allergies.	Full blood count and kidney and liver tests are required before each cycle. Report hearing, tinnitus, nerve, mouth, bowel and fluid-intake changes before dosing; additional tests depend on symptoms and results.	Report symptoms promptly; extra blood or organ tests may be needed.	The team reviews recovery, results and earlier side effects before another dose or phase.	The team follows recovery and response before the next stage, including persistent kidney, hearing, nerve, mouth or bowel effects. Dental, nutrition and swallowing support may continue. Scans, examinations and any radiation-related late-effect tests follow your individual plan; no universal scan or blood tumour-marker schedule is set here.
Treatment and appointments	Confirm the exact regimen, treatment aim, prescription and appointment calendar.	Day 1 includes docetaxel over 60 minutes and cisplatin over 1 hour, plus prescribed fluids, premedication and anti-sickness treatment. Fluorouracil 1,000 mg/m^2 per day runs continuously over 96 hours on Days 1–4; the source also allows a separate 22-hour infusion on each of those four days. Total attendance is longer than the drug infusion alone. Confirm inpatient or outpatient arrangements, any pump changes and final disconnection.	Follow the written support plan and record symptoms and medicines taken.	After induction, the team reviews response and recovery and confirms definitive chemoradiation and/or surgery. The next treatment has its own prescription and calendar; this induction guide does not assign a radiation course or a carboplatin dose.	Record the final treatment dates and the named follow-up team.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Mouth, nutrition and swallowing	Agree oral care and assess weight, intake and swallowing. Arrange dental review before planned head and neck radiation.	Review mouth pain, weight, drinking, swallowing and, during radiation, the treated skin.	Report poor intake, choking, weight loss or inability to drink.	Dietitian and speech and language plans adapt to symptoms and recovery.	Continue dental, nutrition and swallowing care as needed.
Tumour assessment and imaging	Confirm the cancer site, stage and tests relevant to that site.	Discuss symptoms and the agreed treatment aim.	Examinations or scans may be arranged for a particular concern.	Response, recovery and your preferences inform the next treatment decision.	Use your individual scan and clinic plan; this regimen does not set one universal scan or blood tumour-marker schedule.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00315 v7a: official regimen · NCI: head and neck cancers · NCI: oral complications of cancer therapies, patient version · NCI: nutrition during cancer treatment

Tests in your plan

Full blood count

WHEN

At baseline and before every induction cycle.

Checks white cells for infection protection, red cells for oxygen carriage and platelets for clotting.

Kidney function and fluid balance

WHEN

At baseline and before each cycle; extra checks or measured clearance when indicated.

Cisplatin can injure kidneys; results and hydration guide safe prescribing.

Liver function

WHEN

At baseline and before each cycle.

Helps assess whether the medicines, particularly docetaxel, can be given safely.

DPD assessment before fluorouracil

WHEN

Before first exposure unless already tested. Moderate or severe kidney impairment can make a blood-uracil phenotype harder to interpret; the team considers genotype testing where appropriate.

Reduced ability to break down fluorouracil can cause severe or life-threatening toxicity. Normal testing does not exclude every severe reaction.

Hearing assessment

If part of your plan

WHEN

Audiology when indicated; report hearing or tinnitus changes before each dose.

Checks hearing changes or ringing in the ears that cisplatin may cause.

Heart assessment / ECG

If part of your plan

WHEN

ECG for compromised cardiac function; additional assessment for symptoms or clinician concern.

Assesses existing heart problems and helps plan safe treatment; fluorouracil-related chest symptoms need urgent review.

Response assessment

If part of your plan

WHEN

Scans and examinations according to the individual treatment plan.

Helps decide the next treatment stage after induction.

THE WHOLE TREATMENT JOURNEY

Docetaxel 75 / cisplatin 75 / fluorouracil 1,000 · induction TPF: treatment and checks

Induction treatment of stage III/intravenous (IV) non-metastatic squamous cell carcinoma of the head and neck, with a separately agreed definitive chemoradiation and/or surgery plan.

Docetaxel 75 mg/m² and cisplatin 75 mg/m² on Day 1, with fluorouracil 1,000 mg/m²/day on Days 1–4. Repeat every 21 days for 3 cycles, unless progression or unacceptable toxicity changes the plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Treatment phase NCCP	Confirm the full multidisciplinary team (MDT) plan and individual treatment calendar	Docetaxel 75 mg/m² and cisplatin 75 mg/m² on Day 1, with fluorouracil 1,000 mg/m²/day on Days 1–4. Repeat every 21 days for 3 cycles, unless progression or unacceptable toxicity changes the plan.	Follow the actual pump/ disconnect and support plan	After cycle 3, the multidisciplinary team confirms response, recovery and the definitive chemoradiation and/or surgery plan. No automatic radiotherapy date or universal subsequent regimen is authorised by selecting this induction route.	Record the completed medicines and next follow-up plan
Blood and organ checks NCCP	Full blood count, renal and liver profile; clinical status, medicines, allergy and previous treatment review.	Full blood count, renal and liver profile before each cycle; use the current regimen's exact count and organ-function rules.	Report infection, bleeding, reduced urine or dehydration	Repeat or reassess tests if toxicity or recovery changes the plan	Further blood tests and imaging are arranged by the treating team
Support medicines NCCP	Docetaxel premedication is dexamethasone 8 mg orally twice daily for 3 days starting the day before docetaxel; at least 3 doses should precede treatment unless contraindicated. If oral premedication was missed, the source permits dexamethasone 20 mg IV at consultant discretion. This is not automatic permission to proceed. Reconcile the total steroid with antiemetic dexamethasone; do not add both schedules blindly.	Prescribe cisplatin pre- and post-hydration and electrolytes using current renal/cardiac status and local policy. The national source suggests saline/magnesium with optional potassium, but a reference list is not an individual fluid prescription. Monitor intake, urine output, renal function and electrolytes.	granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) support is required in all cycles; NCCP permits standard or pegylated G-CSF. Prescribe product, dose and timing individually; the regimen does not supply one universal injection schedule.	Reconcile all doses, steroid totals and start/stop dates when the phase changes	Review ongoing medicine need with the team

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Mouth, nutrition and swallowing NCCP	Assess mouth, teeth, weight and swallowing needs	Review pain, mouth symptoms, oral intake and hydration	Report inability to drink, severe soreness or weight loss	Dietetic, swallowing and dental support according to need	Continue rehabilitation and late-effect care where needed
Urgent contact NCCP	Save the unit's 24-hour contact and fever instructions	Tell staff about new symptoms or reactions	Seek urgent advice for infection, bleeding, chest pain, severe diarrhoea/vomiting or inability to drink	Do not wait for the next routine appointment if acutely unwell	Keep follow-up and urgent-contact instructions available

What each check is for

1 Induction treatment

Follow your agreed treatment calendar

Shrink and control the cancer before the next definitive local treatment. The team must record the subsequent chemoradiation and/or surgery plan; induction is not the complete treatment pathway.

Ask / act: Confirm active medicines, written supportive-care plan and review criteria.

Sources: NCCP

2 Next-phase / completion review

After the planned course and recovery review

The team checks response and fitness before the next treatment or follow-up stage.

Ask / act: Ask the team to confirm the next phase and actual dates.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-10 · Source reconciled draft; independent clinical, oncology-pharmacy and patient/PPI review pending · SACTCheck v0.84.48